STAKEHOLDER SUMMARY

(ANA-4)

Dataset Used: Adult Psychiatric Morbidity Survey (APMS) 2014

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ANA-1 (Dataset Profiling) · ANA-2 (Demographic Breakdown) · ANA-3 (Regional Mapping).

Project: AI Mental Health Assistant

What the Data Tells Us About Mental Health in England

Summary of Week 1 Analysis Findings for NHS Clinicians

What this is: This summary presents findings from the first week of analysis on the Adult Psychiatric Morbidity Survey (APMS) 2014 — the most comprehensive national study of mental health disorder rates in England. The analysis covered 181 data tables and over 7,500 adult respondents. These findings will directly shape how our AI Mental Health Assistant identifies, prioritises, and responds to people in different risk groups.

Who this is for: NHS clinicians, mental health leads, and service commissioners involved in the AI Assistant project.

Finding 1: Young Women Face a Mental Health Crisis That the Data Cannot Ignore

Across England, **roughly 1 in 4 women (24.2%) currently has a diagnosable common mental disorder**, compared to 1 in 7 men (15.4%). That gap is not surprising to most clinicians — but the scale of it in young women is.

Among women aged 16–24, **36.1% meet the clinical threshold for a common mental disorder** - more than one in three. For men of the same age, the figure is 16.3%. This is the single largest demographic disparity in the dataset.

Group	Men	Women	All Adults
Overall CMD Prevalence	15.4%	**24.2%**	19.9%
Ages 16–24 (Peak gap)	16.3%	**36.1%**	26.2%
CIS-R Score 12+ (clinical threshold)	14.0%	**21.7%**	18.3%

The CIS-R score (the clinical interview scale used in the APMS) places 21.7% of women above the clinical threshold of 12, versus 14.0% of men — confirming that this is not just self-report bias. Young women aged 16–24 are the highest-risk cohort in England.

Finding 2: Where Someone Lives Predicts Their Mental Health Risk

Mental health disorder rates vary significantly by region — by up to 6.3 percentage points between the highest and lowest areas. This is not noise. It reflects real differences in deprivation, employment, service access, and social environment.

The table below shows the full regional picture at a glance:

Region	Anxiety (GAD)	Depression	Any CMD	Risk Tier
South West	5.4%	4.1%	**19.9%**	**High**
North West	7.1%	4.6%	**19.0%**	**High**
London	5.8%	3.3%	**18.9%**	**High**
West Midlands	6.1%	3.8%	**18.6%**	**High**
East Midlands	5.3%	3.3%	**16.7%**	**Medium**
North East	7.2%	3.3%	**16.2%**	**Medium**
Yorkshire & The Humber	4.9%	2.7%	**16.2%**	**Medium**
East of England	6.5%	2.6%	**14.1%**	**Low**
South East	5.4%	2.3%	**13.6%**	**Low**
England (National Average)	**5.97%**	**3.33%**	**17.0%**	**—**

Three patterns stand out:

South West, North West, London, and West Midlands all exceed 18.6%. The South West (19.9%) is the highest in England — which is striking for a largely rural region. Geographic isolation, reduced NHS service density, and seasonal employment instability likely drive this. Clinicians in these areas should expect a higher volume of CMD presentations.

North East alert — the North East sits in the medium-risk tier overall (16.2%), but records the **highest anxiety (GAD) rate in England at 7.2%**. A region's headline score can mask serious disorder-specific peaks. Anxiety-specific services in the North East are under particular pressure.

South East and East of England are the lowest-burden regions (13.6% and 14.1%). Higher household incomes, lower unemployment, and better primary care access are the likely explanatory factors. These regions still carry significant CMD burden — but at meaningfully lower rates than the rest of England.

Finding 3: Anxiety is the Dominant Disorder — But the Mix Varies by Region

Nationally, Generalised Anxiety Disorder (GAD) is the most common specific disorder at 6.0% of all adults, followed by depression at 3.3%. But the disorder mix is not uniform across regions:

- The North West has both the highest depression rate (4.6%) and the highest phobia rate (3.1%) in England.
- The East Midlands has the highest OCD rate outside London (2.7%) — nearly double the national average.
- London's CMD-NOS rate (9.4%) — broadly, presentations that do not meet a single diagnostic threshold — is the highest nationally, pointing to complex, mixed-presentation cases.
- The South West's high overall rate is driven primarily by depression (4.1%) and phobias (2.8%), not anxiety.

This matters clinically: a one-size-fits-all screening tool will miss these regional patterns. An AI system trained only on national averages would under-prioritise anxiety in the North East and over-prioritise it in Yorkshire.

What This Means for the AI Mental Health Assistant

These findings are not just statistics — they are directly translated into how the AI tool will behave. The table below summarises the five key data-to-action translations:

Finding	How the AI Assistant will use it
Young women aged 16–24 are the highest-risk cohort	Screen more sensitively for CMD when user profile matches this cohort; prompt earlier escalation to a clinician
South West, North West, and London exceed 18% CMD	Apply higher baseline risk thresholds for users in these regions; surface region-specific NHS service links
North East has the highest anxiety (GAD) rate despite a medium overall score	Trigger anxiety-specific assessment pathways for North East users regardless of overall CMD score
Generalised Anxiety Disorder is the most prevalent specific disorder nationally (6.0%)	Weight GAD screening questions more heavily in initial triage; include PHQ/GAD-7 prompts by default
Depression rates vary up to 2× across regions (2.3%–4.6%)	Personalise depression resource recommendations and referral urgency by user region

The bottom line for clinicians

The AI Assistant will not treat every user the same. It will adjust its screening sensitivity, resource suggestions, and escalation thresholds based on the user's age, gender, and region — because the data shows clearly that risk is not distributed evenly. A 20-year-old woman in the North West is not the same clinical picture as a 50-year-old man in the South East, and the tool will reflect that.

Week 2 analysis will incorporate ONS time-series data and PHQ-9/GAD-7 scored datasets to add the temporal and severity dimensions to this picture.